

AI Interview Assist

Interview Performance Report

Interview Information

Interview ID	6aa93fc7a759fa7d0d4c8d87
Job Role	ICU Nurse
Experience Level	Mid Level
Interview Type	Mixed
Difficulty	Medium
Total Questions	5

Overall Performance

81.0%

OVERALL PERFORMANCE

Very Good

Average Score: 8.1/10

Total Score 40.5 / 50	Average 8.1 / 10	Percentage 81.0%
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AI Performance Analysis

Summary

Interview completed successfully. The overall score is calculated from your evaluated answers.

Strengths

Not available

Weaknesses

Not available

Recommendations

Not available

Technical Skill Assessment

Assessment is based on the scores received for the evaluated answers.

Readiness Assessment

Continue practicing the areas where your score was lower.

Question-wise Performance

Question 1

6.0/10

Question

How do you systematically approach titrating multiple vasoactive drips, such as norepinephrine and vasopressin, in a patient exhibiting septic shock with refractory hypotension?

Candidate Answer

- **Target MAP:** ≥ 65 mmHg. - **Norepinephrine:** Titrate to achieve the MAP target. - **Vasopressin:** Add if hypotension persists despite norepinephrine. - **Monitor:** BP/MAP, urine output, lactate, and tissue perfusion. - Use the **lowest effective doses** and reassess frequently.

Correctness	Partially Correct	Relevance	Highly Relevant
Technical Accuracy	Average	Score	6.0/10

Strengths

- Accurately identifies the target Mean Arterial Pressure (MAP) of ≥ 65 mmHg for septic shock.
- Correctly designates norepinephrine as the primary vasoactive agent and vasopressin as an adjunctive agent for refractory hypotension.
- Includes essential clinical monitoring parameters beyond MAP, such as urine output, lactate levels, and tissue perfusion.

Weaknesses

- Fails to mention that vasopressin is typically administered as a fixed-dose infusion (e.g., 0.03 units/min) rather than titrated like norepinephrine.
- The response is overly brief and lacks detailed, step-by-step nursing workflow mechanics for managing multiple concurrent drips.

Missing Concepts

- Fixed-dose administration of vasopressin versus variable-rate titration of norepinephrine.
- Importance of an arterial line for continuous, accurate blood pressure monitoring during active titration.
- Central venous catheter requirement for safe administration of multiple high-risk vasopressors.
- Systematic order of weaning/de-escalation once stability is achieved.

AI Feedback

Your answer demonstrates a solid foundational understanding of the core concepts in managing septic shock, including first-line agents, MAP targets, and perfusion monitoring. However, for a mid-level ICU nurse, the answer lacks key clinical nuances. Specifically, vasopressin is standardly given as a fixed-dose non-titratable drip rather than a titrated drug. To improve, discuss specific practical protocols such as using invasive arterial lines for real-time monitoring, ensuring central line access, and explaining how you manage titrations and weaning in tandem.

Question

A mechanically ventilated patient suddenly triggers a high peak pressure alarm and shows a drop in oxygen saturation. What are your immediate troubleshooting steps to identify and manage the cause?

Candidate Answer

- **Assess the patient first:** Check SpO₂, chest movement, breath sounds, and vital signs; give 100% O₂ if needed. - **Check the ventilator/tubing:** Look for kinks, obstruction, disconnection, or water in the circuit. - **Check the ETT:** Ensure it is not blocked by secretions or kinked; suction if indicated. - **Consider common causes:** Bronchospasm, mucus plug, pneumothorax, or decreased lung compliance. - **If unstable:** Disconnect from the ventilator and manually ventilate with a bag-valve device while urgently seeking help. - **Treat the identified cause** and reassess airway pressure and oxygenation.

Correctness	Correct	Relevance	Highly Relevant
Technical Accuracy	Excellent	Score	9.0/10

Strengths

- Prioritizes immediate patient assessment and safety before troubleshooting equipment.
- Correctly identifies the critical step of disconnecting from the ventilator and manually ventilating with a Bag-Valve-Mask (BVM) on 100% O₂ if the patient is unstable.
- Systematically addresses circuit, endotracheal tube (ETT), and patient-level causes aligned with standard ICU nursing protocols (e.g., DOPE mnemonic).
- Includes key differential diagnoses such as pneumothorax, bronchospasm, and mucus plugging.

Weaknesses

- Lacks specific mention of using a suction catheter as a diagnostic tool to assess ETT patency (i.e., whether the catheter passes freely).
- Does not detail differentiating peak inspiratory pressure (PIP) from plateau pressure (Pplat) to separate airway resistance issues from lung compliance issues.

Missing Concepts

- Differentiating between Peak Inspiratory Pressure (PIP) and Plateau Pressure (Pplat) to narrow down resistance versus compliance causes.
- Using a suction catheter test to quickly confirm if the endotracheal tube is obstructed or bitten.

AI Feedback

The candidate provided a structured, highly accurate, and patient-centered answer that demonstrates solid clinical judgment suitable for a mid-level ICU nurse. Emphasizing manual ventilation with a BVM during instability shows strong safety awareness. To make the answer outstanding, the candidate could mention assessing plateau pressure to differentiate compliance issues (like pneumothorax or ARDS) from resistance issues (like bronchospasm or biting the tube).

Question

What are the early clinical signs and laboratory indicators of Disseminated Intravascular Coagulation (DIC) in an ICU patient, and what immediate nursing actions should be taken?

Candidate Answer

Early signs - Oozing/bleeding from IV or catheter sites, gums, or wounds - Petechiae, bruising, or hematuria - Hypotension, tachycardia, or signs of poor organ perfusion - Altered mental status or reduced urine output #### Laboratory indicators - ↓ Platelet count - ↑ PT/INR and aPTT - ↑ D-dimer/FDP - ↓ Fibrinogen - Falling hemoglobin/hematocrit #### Immediate nursing actions - ****Notify the ICU/medical team immediately.**** - Monitor vital signs, bleeding, urine output, and neurological status closely. - Minimize invasive procedures and avoid unnecessary IM injections. - Obtain/monitor ordered coagulation and blood tests. - Maintain IV access and prepare for prescribed blood products/treatment. - Treat and support management of the ****underlying cause****, such as sepsis.

Correctness	Correct	Relevance	Highly Relevant
Technical Accuracy	Excellent	Score	10.0/10

Strengths

- Accurately identified early clinical signs of both bleeding and microvascular thrombosis (e.g., oozing from line sites, petechiae, decreased urine output, altered mental status).
- Correctly outlined all classic laboratory indicators of DIC including decreased platelets, prolonged PT/aPTT, elevated D-dimer/FDP, and low fibrinogen.
- Provided prioritized, actionable nursing interventions such as notifying the medical team, minimizing invasive procedures, maintaining access, preparing for blood transfusion, and addressing the underlying cause.

Weaknesses

Not available

Missing Concepts

- Mentioning specific blood components tailored to lab values (e.g., Cryoprecipitate specifically for low fibrinogen, FFP for clotting factors, platelets for thrombocytopenia) would add extra clinical depth.

AI Feedback

Your answer is exceptionally strong, accurate, and structured. You covered clinical signs, laboratory markers, and critical nursing interventions thoroughly, demonstrating a solid understanding appropriate for a mid-level ICU nurse.

Question

Describe a scenario where you detected a subtle change in a critically ill patient's neurological status. How did you evaluate the change and communicate your findings to the medical team?

Candidate Answer

Example scenario I noticed that a previously alert ICU patient became **slightly confused and less responsive to verbal commands**. I reassessed the patient using **GCS, pupil size/reactivity, vital signs, blood glucose, and oxygen saturation**, and compared the findings with the previous assessment. I immediately **informed the medical team using SBAR**, clearly stating the baseline status, the new neurological changes, assessment findings, and the time of onset. I continued close neurological monitoring and prepared for further investigations as ordered.

Correctness	Correct	Relevance	Highly Relevant
Technical Accuracy	Good	Score	7.0/10

Strengths

- Correctly identifies essential assessment parameters including GCS, pupil reactivity, vital signs, and important rule-outs like blood glucose and oxygen saturation.
- Utilizes a structured communication framework (SBAR) to report changes effectively to the medical team.
- Emphasizes comparing current findings against the patient's baseline neurological status.

Weaknesses

- The response reads as a general procedure or textbook template rather than a detailed, real-life clinical scenario.
- Lacks specific clinical context, such as the patient's underlying diagnosis, focal neurological signs, or specific diagnostic orders anticipated (e.g., STAT head CT, ABG).

Missing Concepts

- Assessment for specific focal neurological deficits (e.g., NIHSS components, limb strength/asymmetry).
- Anticipating specific diagnostic interventions like a STAT non-contrast head CT or arterial blood gas analysis.

AI Feedback

Your answer demonstrates sound clinical judgment and correct utilization of the SBAR tool and neurological assessment protocols (including ruling out hypoglycemia and hypoxia). However, as a mid-level ICU nurse, you should frame your response around a detailed, specific patient scenario to showcase practical experience rather than giving a generalized, high-level summary.

Question

If a patient undergoing Continuous Renal Replacement Therapy (CRRT) experiences a cardiac arrest, what are your immediate priorities regarding both resuscitation and managing the CRRT circuit?

Candidate Answer

- **Start CPR immediately** and follow the ACLS/code protocol; call for the resuscitation team. - Give **100% oxygen** and assess airway, breathing, and circulation. - **Pause CRRT** if necessary to facilitate resuscitation and prevent circuit-related complications. - **Do not return the circuit blood** if it is unsafe or if the circuit is suspected to be contaminated/clotted. - Check the CRRT circuit for **air, disconnection, bleeding, or clotting** and clamp lines as appropriate. - After ROSC, reassess the patient and **restart/reconnect CRRT safely** according to the ICU/nephrology protocol.

Correctness	Correct	Relevance	Highly Relevant
Technical Accuracy	Good	Score	8.5/10

Strengths

- Prioritizes immediate high-quality CPR and activation of the ACLS protocol without delay.
- Appropriately addresses CRRT circuit management, including pausing the pump and clamping lines to prevent complications.
- Correctly identifies safety precautions regarding not returning circuit blood if clotting or contamination is suspected.

Weaknesses

- Does not address vascular access management specifically regarding ACLS medication administration using CRRT lumens versus dedicated IV/CVC access.

Missing Concepts

- Precautions regarding vascular access during code (avoiding CRRT lumens for ACLS medications due to dead space, heparin/citrate content, or air embolism risk unless no other access exists).
- Identifying CRRT-related reversible causes of cardiac arrest during the code (e.g., severe electrolyte shifts like hyperkalemia/hypocalcemia or extreme volume depletion/overload).

AI Feedback

The candidate demonstrated a strong, safe, and logical approach to managing a cardiac arrest in a patient on CRRT. They correctly prioritized standard ACLS resuscitation alongside essential CRRT circuit safety steps. To elevate the response to an expert level, the candidate should discuss vascular access nuances during resuscitation and mention evaluating CRRT-specific causes for the arrest (e.g., electrolyte or fluid balance shifts).

AI Interview Assist

This report was generated based on the candidate's interview responses and AI evaluation.